

Public Health Emergency Operations Center
"COUSP DRC"

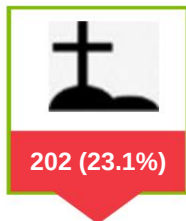
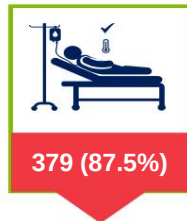
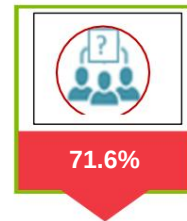
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°33/MVB_16/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (33)	• Ituri (21/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 16, 2026
Publication date	June 17, 2026

Total confirmed cases
for the 3 provincesCumulative deaths
among confirmed cases
and case fatality ratePatients in
isolation -
hospitalizationCumulative number
of recoveriesSuspected cases of
the dayContact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as the cases that escaped inclusion in the overall count.

Suspicious deaths occurring in the community are subject to investigation. further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

- **Forty (40) new confirmed cases including 4 deaths**, were reported in the provinces of **Ituri (32 including 1 death) and North Kivu (8 including 3 deaths)** at the end of the day on June 16, 2026. These confirmed cases came from the health zones of **Rwampara (13), Bunia (11), Mongbwalu (4), Tchomia (3), Fataki (1)** in the province of Ituri as well as those of **Butembo (4), Katwa (2), Oicha (1) and Musienene (1)** in the province of North Kivu.
- Two (2) new health zones have been affected, namely those of **Fataki** (Ituri province) and **Musienene** (North Kivu).
- **Eighteen (18) patients** with Ebola virus disease have been declared cured after obtaining negative control tests.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements toward Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

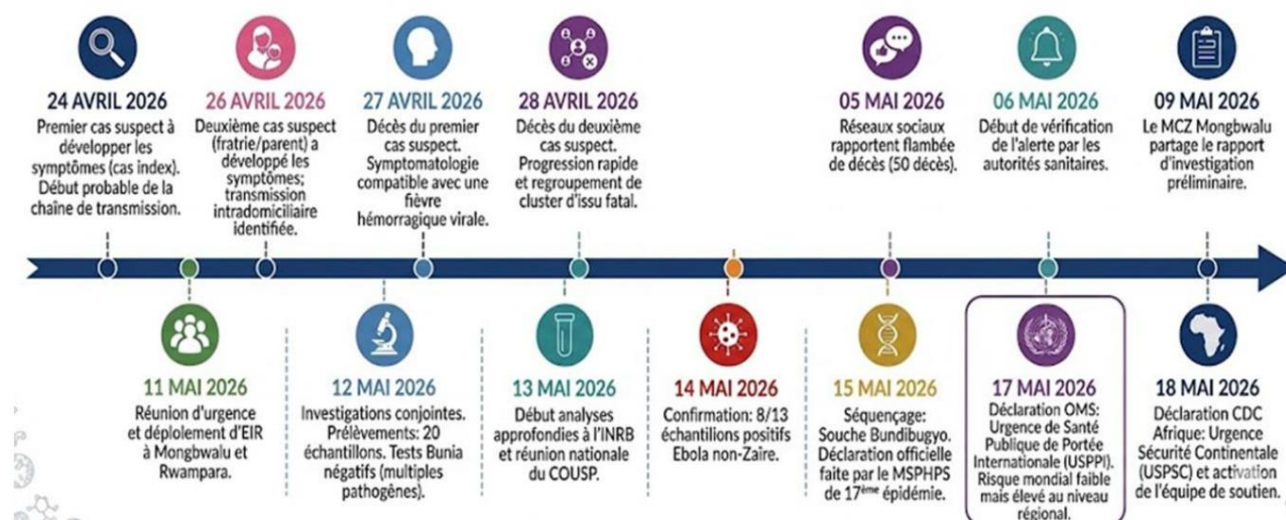


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 16, 2026

Table 1. Distribution of confirmed cases and deaths by affected province as of June 16, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	799	158	19.8%	21 out of 36 (58.3%)	32
North Kivu 1	73	43	58.9%	11 out of 34 (32.3%)	8
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	875	202	23.1%	33 out of 104 (31.7%)	40

As of June 16, 2026, two new health zones were affected: Fataki in Ituri and Musienene in North Kivu. The number of affected health zones increased from 31 to 33 out of 104 (section 3.1). Ituri province remains the epicenter, accounting for 91.3% of confirmed cases reported across the province.

The three affected provinces. North Kivu province remains the most affected in terms of fatality rate (58.9%), while Ituri province shows a low fatality rate (19.8%) compared to North Kivu.

despite the significant number of deaths recorded. On the other hand, the province of South Kivu has not reported any confirmed cases since May 26, 2026 (Table 1).

3.2 Distribution by health zone (Cumulative as of 16/06/2026)

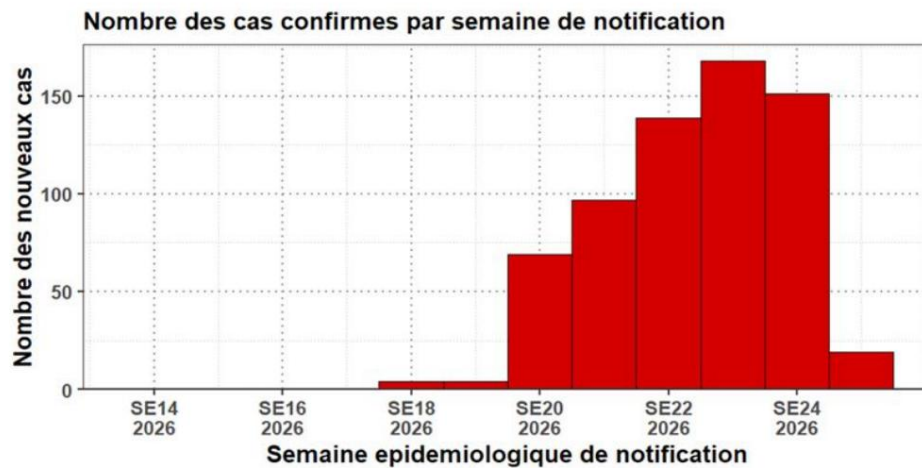
- **Province of Ituri (799 cases):** Bunia (226), Rwampara (172), Mongbwalu (189), Nyankunde (45), Nizi (16), Bambu (7), Komanda (7), Lita (9), Mangala (5), Tchomia (5), Kilo (4), Damas (4), Rimba (3), Aru (3),

1 The data from North Kivu have been harmonized and distributed by health zone bringing the cumulative number of confirmed cases to 73 (43 deaths included).

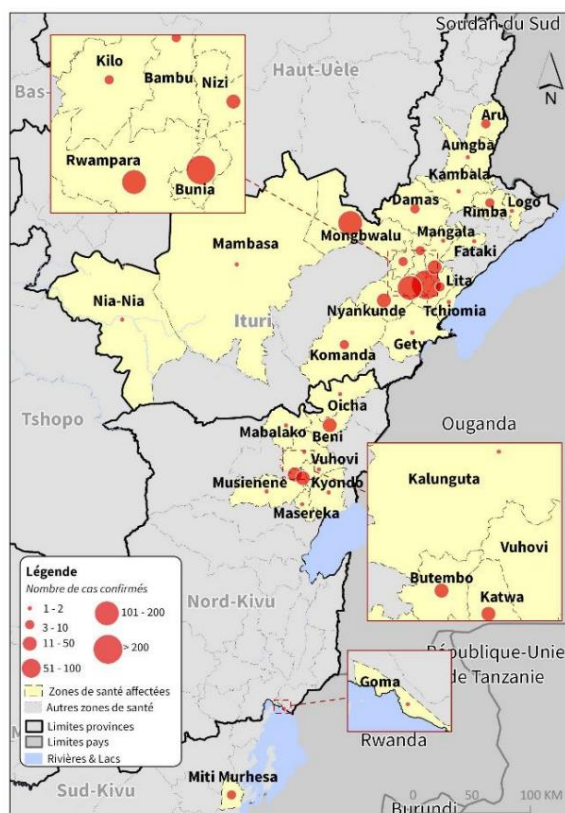
Aungba (2), Mambasa (2), Logo (2), Fataki (1), Gety (1), Kambala (1) and Nia-Nia (1 case). (Ninety-four (94) additional cases are being ventilated.

- **North Kivu Province (73 cases):** Katwa (23), Butembo (24), Beni (14), Oicha (3), Kyondo (2), Kalunguta (2), Goma (1), Masereka (1), Vuhovi (1), Mabalako (1) and Musienene (1).
- **South Kivu Province (3 cases) :** Miti-Murhesa (3).

3.3 Temporal Analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting continuous transmission of the disease within the community. Rapid geographical expansion is also observed. The epidemic remains possible if public health actions are not implemented quickly.



3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 16, 2026

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Workshop preparations for reviewing the initial actions of the response to Ebola virus disease.
- Holding of the coordination meeting on monitoring the activities carried out in the field by the partners technical and financial aspects across the pillars of the response.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 16, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		391	332	14	737
Alerts investigated		129	332	14	475
Investigation rate		33.0%	100.0%	100.0%	64.5%
Alerts confirmed	Alive	93	44	9	146
	Deceased	31	15	0	46
Total suspected cases for today		124	59	9	192

By the end of the day on June 16, 2026, **737 alerts** had been reported in the three affected provinces, **475 (64.5%)** of these alerts were investigated, **192 suspected cases (46 deaths)** were identified.

Table 3. Contact tracing of confirmed cases as of June 16, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	5,309	3,856	72.6%
North Kivu	1,467	974	66.4%
South Kivu	80	80	100.0%
Total	6,856	4,910	71.6%

- Continued in-depth investigations and exhaustive listing and monitoring of contacts around confirmed cases in the ZS (narratives of confirmed cases, number of contacts listed and monitored per case).
- 371 new contacts were listed in 7 health zones in Ituri province, including 100 in Rwampara and 86 in Bunia 81 in Mongbwalu, 54 in Nyankunde, 34 in Aungba, 14 in Lita and 2 in Mambasa.

Update on checkpoint and point of entry (PoC/PoE) activities

- Two (2) alerts intercepted, notified and validated in Mudzibala from Mabanga (Nizi Health Zone),
These people were all referred to the Bunia General Referral Hospital.
- Two alerts (lifeless bodies) intercepted, notified after verification of documents, the bodies were released to be transported to the cemetery (swabs carried out and negative results for the MVE).

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 16, 2026

Indicators	Results
Proportion of activated PoCs	80.0% (44/55)
Number of travelers who passed through, % of travelers	54,660 98.0%
screened, % of travelers who washed their hands, % of travelers made aware of the risks	97.5% 99.0%
Number of alerts notified	4
Percentage of alerts investigated or bodies swabbed within 24 hours	100.0%
Number of validated alerts	4
% of results available in 24 hours	50.0%
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Infection Prevention and Control (IPC)

- **Ituri:** Two (2) briefings of service providers organised in the Rwampara Health Zone (AS Ngona and AS Shari) on an IPC assessment; provision of IPC kits in the Rwampara Health Zone; opening of an IPC ring around cases notified in the Nyankunde and Mongbwalu Health Zones in the last 48 hours (5 households identified, including 3 in Mongbwalu and 2 in Nyankunde).
- **North Kivu:** Start of the 2-day IPC briefing for health providers at MONUSCO clinics in Goma; assessment of 5/7 staff exposed to a confirmed case of Ebola at CH Félicité with a high degree of exposure; assessment of 23 staff exposed in Beni, 12 of whom are classified as high risk; 126 providers briefed on the various IPC themes in the context of the Ebola epidemic during the support of 34 health centers in the health zones of Beni, Butembo and Katwa; supervision of CH LA VERITE which received 5 confirmed positive cases; decontamination around the confirmed cases in Butembo and Katwa; decontamination of 1 out of 2 health centers (CH DON DIVIN) in the health zone of Katwa; evaluation score card of 12 ESS, including 10 in Beni with an average of 55.7% and 2 in Katwa including HGR KITATUMBA with 42%, dispensaire Félicité (24.6%); provision of PCI inputs to CH Wanamahika in the ZS of Katwa.
- **South Kivu:** Scorecard evaluation of 5 ESSs, including 4 in the Katana health zone and 1 in the Mitimurhesa health zone; decontamination of 8 households and 8 latrines around 8 suspected cases in the Katana health zone (5 in Kalagane and 3 in Chiranga).

4.4 Holistic care

- 18 new recoveries, including a 16-month-old child. Among these recoveries, 3 are PPL.
- Assistance with the delivery of a confirmed case.
- Operationalization of the CTE IST Nyankunde.
- Readmission of a patient previously classified as a non-case (orphan) with fever and syndrome hemorrhagic.

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 16, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	321	45	10	376
Total admissions (24h)	47	11	9	67
Exits (24h)				
Deceased (Suspects/Confessions)	8	1	0	9
No case	23	9	4	36
Cured	18	0	0	18
Escapees (Suspects/Confessions)	1	0	0	1
Transferred to the HGR	0	0	0	0
Total outflows	50	10	4	64
Patients in isolation (End of Day),	318	46	15	379
including confirmed cases	138	13	1	152
(NC+AC) and suspected cases	180	33	14	227
Overall occupancy rate	96.4%	64.8%	46.9%	87.5%

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 16, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	2 (28.5%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	34 (25.9%)	5 (41.7%)	1 (100%)
New suspected cases that benefited from SMSPS interventions	9 (19.1%)	5 (62.5%)	2 (100%)
Suspected cases under follow-up that have received interventions SMSPS	36 (27%)	16 (28.1%)	6 (100%)
Laboratory results have been announced.	7	0	11
Including positive ones	3	0	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	2	9
Number of accompanying persons at the CTE who benefited from interventions SMSPS	44	4	3
Number of PPLs who benefited from SMSPS interventions	28	8	15
Number of household and community members who benefited from SMSPS interventions	69	20	8
Number of EDS supported	0	0	0

4.5 Continuity of care

- Monitoring of drug supplies in the health areas of the Bunia health zone.
- Holding of the preparatory meeting in anticipation of the training of midwives in the areas of health of Rwampara and Nyankunde.

4.6 Risk communication and community engagement

North Kivu: 33,708 people reached in 15,687 households by 2,005 community outreach teams, representing 73% of the target of 46,000 people and 68% of the 23,000 targeted households, reflecting a 9% increase in coverage compared to the previous day; 1,042 people reached in schools, universities, businesses, sports facilities, and at the Health Center, promising to pass on the messages to their families, while 488 people were reached on hand hygiene during the Day of the African Child; 18 local radio stations covering 72% of the population, 3 communication formats validated with 91% clarity; strategic advocacy efforts were carried out, one with the mayor of Kimemi to facilitate access to outlying neighborhoods and another with grassroots leaders to strengthen their active involvement in awareness-raising; 144 members of the AS Matanda CAC (101 men and 43 women) were sensitized on barrier measures and the reporting of community alerts, creating a local surveillance network capable of identifying any suspected case early in this strategic area.

South Kivu: Community resistance was lifted around the confirmed escaped case, who agreed to return for sampling (Mitimurhesa health zone); mass awareness campaign with 436 schoolchildren (253 boys and 183 girls) from the Les Bâtisseurs school complex on the importance of handwashing; educational talk with 28 mothers during the ANC at the Kavumu Health Center on Ebola virus disease prevention measures; 465 households visited, 1810 people sensitized on prevention measures and signs of Ebola virus disease, 176 community alerts reported, 22 leaders engaged; 2 radio stations broadcast messages about Ebola virus disease in the Katana and Mitimurhesa health zones.

Ituri: Organization of the large motorized caravan to accompany the unloading of 7 recovered patients and to raise awareness about the effectiveness of care in the Rwampara Ebola Treatment Centers (ETCs). On the occasion of the discharge of the recovered patients, the Chief of the Tsere Group (in the Bahema chiefdom of Irumu), the Chief Medical Officer of the area, and some recovered patients raise awareness among the population about preventive measures for Ebola with a focus on joining the ETC; home visits to 7,050 households by 470 community health workers with messages on Ebola prevention measures and the reporting of alerts (deaths and survivors) affecting 12,285 people (5,029 men and 7,256 women) in the Rwampara health zone; 9 community health workers conducted educational talks in 9 households and reached 190 people (59 men and 131 women); Awareness-raising of 226 people on compliance with Ebola prevention measures, care, community alerts and EDS; community dialogue with 30 people (17 men and 13 women) discussing community involvement in the Ebola response in the Mongbwalu Health Zone; briefing of 132 service providers on Community-Based Surveillance, the alert reporting process, monitoring of suspected cases, and data collection and reporting.

community feedback in the health zones of Mongbwalu, Rwampara, Bunia; training of 77 people in the Bunia health zone on awareness, risk communication and community engagement, preventive measures for Ebola Virus Disease (EVD), the importance of EDS and household decontamination; dissemination of spots and messages in 18 media outlets on EVD preventive measures.

Support to other pillars : Surveillance : Support for the investigation in the Bora Uzima AS around an alert.

PCI : Preparing a family for enrollment in the AS Muzimari EDS. Result: EDS completed; **Support :** Organizing a large motorized caravan to accompany the unloading of 7 recovered patients and raise awareness about the effectiveness of the care provided at the Rwampara CTE.

4.7 Logistics

- Reception of a batch of medicines (support from UG-PDSS) for the health zones of Bunia, Ariwara, Damas, Kilo, Bambu, Mongbwalu.
- Delivery of IPC medicines and supplies to the health zones of Mongbwalu, Komanda, Nia-Nia, Damas and Bunia.
- Delivery of medicines to the CTE/HGR Bunia with support from ALIMA and IMC.

- Supervisory visits to the BCZ Bunia depot and the CTE CME Bunia (place of storage of medicines and PCI inputs).
- Site visits to two large garages in the city of Bunia (purpose: potential selection) (for the maintenance of the response vehicle fleet).

4.8 Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.9 PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : continued briefings on PSEA and good conduct in the community.

South Kivu: PSEA awareness-raising of 26 hygienists (15 women) and 64 community relays (15 women) involved in the active search for EVD cases.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of construction of CTE planned
3 Weakness in contact tracing (overall rate 71.6%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Insufficient supply of Essential Generic Medicines (EGMs) in targeted General Referral Hospitals	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the province from South Kivu	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action	Responsible pillar	Due date
1 Strengthen awareness-raising and community engagement activities of local leaders	CREC	Continue
2 Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC Day 3	
3 Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC Day 3	
4 Supervised support for national and provincial coordination in the technical pillars	Coordination	Continue
5 Catch-up in contact tracing in Ituri remains weak and is declining in North Kivu	Monitoring	Day 7
6 Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7 Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8 Plan for catching up on samples awaiting analysis in North Kivu Laboratory		Day 2

Discharge ceremony for patients cured of Ebola virus disease (EVD) from the Ebola Treatment Center/General Referral Hospital (ETHC) in the Rwampara health zone, Ituri province.



Handover of starlink kits and tablets to strengthen data collection in the Komanda health zone, Ituri province.



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